Taipei Veterans General Hospital

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Sample Information

Patient Name: 王水治
Gender: Female
ID No.: J221144777
History No.: 36189512
Age: 62

Ordering Doctor: DOC6420A 蔡明軒
Ordering REQ.: 0CNTFZF
Signing in Date: 2023/07/27

Path No.: M112-00190
MP No.: F23058
Assay: Oncomine Focus Assay
Sample Type: FFPE
Block No.: S112-34988A+B
Percentage of tumor cells: 30%

Reporting Doctor: DOC5466K 葉奕成 (Phone: 8#5466)

Note:

Sample Cancer Type: Non-Small Cell Lung Cancer

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Report Highlights

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Relevant Non-Small Cell Lung Cancer Variants

Gene	Finding	Gene	Finding
ALK	None detected	NTRK1	None detected
BRAF	None detected	NTRK2	None detected
EGFR	***EGFR exon 19 deletion, EGFR p.(T790M) c.2369C>T, EGFR p.(C797S) c.2389T>A***	NTRK3	None detected
ERBB2	None detected	RET	None detected
KRAS	None detected	ROS1	None detected
MET	None detected		

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003). The content of this report has not been evaluated or approved by FDA, EMA or other regulatory agencies.

Relevant Biomarkers

Tier	Genomic Alteration	Relevant Therapies (In this cancer type)	Relevant Therapies (In other cancer type)	Clinical Trials
IA	*EGFR exon 19 deletion* epidermal growth factor receptor Allele Frequency: 54.82%	**bevacizumab* + erlotinib** [2] **erlotinib + ramucirumab** [1] **osimertinib** [1, 2] atezolizumab + bevacizumab + chemotherapy gefitinib + chemotherapy	None	12
IA	*EGFR p.(T790M) c.2369C>T* epidermal growth factor receptor Allele Frequency: 38.62%	**osimertinib** [1, 2] atezolizumab + bevacizumab + chemotherapy	None	7
IIC	*PIK3CA p.(H1047R) c.3140A>G* phosphatidylinositol-4,5-bisphosphate 3-kinase catalytic subunit alpha Allele Frequency: 31.15%	None	**alpelisib + hormone therapy** [1, 2]	1
IIC	*EGFR p.(C797S) c.2389T>A* epidermal growth factor receptor Allele Frequency: 40.16%	None	None	5

Public data sources included in relevant therapies: FDA[1], NCCN, **EMA**[2], ESMO
Tier Reference: Li et al. *Standards and Guidelines for the Interpretation and Reporting of Sequence Variants in Cancer: A Joint Consensus Recommendation of the Association for Molecular Pathology, American Society of Clinical Oncology, and College of American Pathologists.* J Mol Diagn. 2017 Jan;19(1):4-23.
*** Includes biosimilars/generics**

Alerts informed by public data sources: ⊘ Contraindicated, Resistance

EGFR p.(T790M) c.2369C>T	⊘ **gefitinib** [2] afatinib, dacomitinib, erlotinib, gefitinib

Public data sources included in alerts: FDA[1], NCCN, **EMA**[2], ESMO

Prevalent cancer biomarkers without relevant evidence based on included data sources
CDK4 amplification

Variants (Exclude variant in Taiwan BioBank with >1% allele frequency)

DNA Sequence Variants

Gene	Amino Acid Change	Coding	Variant ID	Locus	Allele Frequency	Transcript	Variant Effect	Coverage
PIK3CA	p.(H1047R)	c.3140A>G	COSM775	chr3:178952085	31.15%	NM_006218.4	missense	2000
EGFR	p.(L747_P753delinsS)	c.2240_2257delTAAGAGAAGCAACATCTC	COSM12370	chr7:55242469	54.82%	NM_005228.5	nonframeshift Deletion	1959
EGFR	p.(T790M)	c.2369C>T	COSM6240	chr7:55249071	38.62%	NM_005228.5	missense	1999
EGFR	p.(C797S)	c.2389T>A	COSM6493937	chr7:55249091	40.16%	NM_005228.5	missense	1997

Copy Number Variations

Gene	Locus	Copy Number
CDK4	chr12:58142052	48.17

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Biomarker Descriptions

CDK4 (cyclin dependent kinase 4)

Background: The CDK4 gene encodes the cyclin-dependent kinase 4 protein, a homologue of CDK6. Both proteins are serine/threonine protein kinases that are involved in the regulation of the G1/S phase transition of the mitotic cell cycle[1,2]. CDK4 kinase is activated by complex formation with D-type cyclins (e.g., CCND1, CCND2, or CCND3), which leads to the phosphorylation of retinoblastoma protein (RB), followed by E2F activation, DNA replication, and cell-cycle progression[3]. Germline mutations in CDK4 are associated with familial melanoma[4,5,6].

Alterations and prevalence: Recurrent somatic mutations of CDK4 codon K22 and R24 are observed in melanoma (1-2%) and lung cancer (approximately 0.1%). Codons K22 and R24 are necessary for binding and inhibition by p16/CDKN2A[7,8,9]. CDK4 is recurrently amplified in several cancer types, most notably in sarcomas (15-20%), glioma (10-15%), adrenocortical carcinoma (5%), lung adenocarcinoma (5%), and melanoma (3%)[10,11,12,13].

Potential relevance: Currently, no therapies are approved for CDK4 aberrations. Amplification of region 12q14-15, which includes CDK4, is useful as an ancillary diagnostic marker of atypical lipomatous tumor/welldifferentiated liposarcoma (ALT/WDLS)[14]. Small molecule inhibitors targeting CDK4/6 including palbociclib (2015), abemaciclib (2017), and ribociclib (2017), are FDA approved in combination with an aromatase inhibitor or fulvestrant for the treatment of hormone receptor-positive, HER2-negative advanced or metastatic breast cancer.

EGFR (epidermal growth factor receptor)

Background: The EGFR gene encodes the epidermal growth factor receptor (EGFR) tyrosine kinase, a member of the ERBB/human epidermal growth factor receptor (HER) family. In addition to EGFR/ERBB1/HER1, other members of the ERBB/HER family include ERBB2/HER2, ERBB3/HER3, and ERBB4/HER4[15]. EGFR ligand induced dimerization results in kinase activation and leads to stimulation of oncogenic signaling pathways including the PI3K/AKT/MTOR and RAS/RAF/MEK/ERK pathways. Activation of these pathways promote cell proliferation, differentiation, and survival[16,17].

Alterations and prevalence: Recurrent somatic mutations in the tyrosine kinase domain (TKD) of EGFR are observed in approximately 10-20% of lung adenocarcinoma, and at higher frequencies in never-smoker, female, and Asian populations[10,11,12,18]. The most common mutations occur near the ATP-binding pocket of the TKD and include short in-frame deletions in exon 19 (EGFR exon 19 deletion) and the L858R amino acid substitution in exon 21[19]. These mutations constitutively activate EGFR resulting in downstream signaling, and represent 80% of the EGFR mutations observed in lung cancer. A second group of less prevalent activating mutations include E709K, G719X, S768I, L861Q, and short in-frame insertion mutations in exon 20[20,21,22,23]. EGFR activating mutations in lung cancer tend to be mutually exclusive to KRAS activating mutations[24]. In contrast, a different set of recurrent activating EGFR mutations in the extracellular domain include R108K, A289V and G598V and are primarily observed in glioblastoma[13,19]. Amplification of EGFR is observed in several cancer types including 30% of glioblastoma, 12% of esophageal cancer, 10% of head and neck cancer, 5% of bladder cancer, and 5% of lung squamous cell carcinoma[10,11,12,13,25]. Deletion of exons 2-7, encoding the extracellular domain of EGFR (EGFRvIII), results in overexpression of a ligand-independent constitutively active protein and is observed in approximately 30% of glioblastoma[26,27,28].

Potential relevance: Approved first-generation EGFR tyrosine kinase inhibitors (TKIs) include erlotinib[29] (2004) and gefitinib[30] (2015), which block the activation of downstream signaling by reversible interaction with the ATP-binding site. Although initially approved for advanced lung cancer, the discovery that drug sensitivity was associated with exon 19 and exon 21 activating mutations allowed first-generation TKIs to become subsequently approved for front-line therapy in lung cancer tumors containing exon 19 or exon 21 activating mutations. Second-generation TKIs afatinib[31] (2013) and dacomitinib[32] (2018) bind EGFR and other ERBB/HER gene family members irreversibly and were subsequently approved. First- and second-generation TKIs afatinib, dacomitinib, erlotinib, and gefitinib are recommended for the treatment NSCLC harboring EGFR exon 19 insertions, exon 19 deletions, point mutations L861Q, L858R, S768I, and codon 719 mutations, whereas most EGFR exon 20 insertions, except p.A763_Y764insFQEA, confer resistance to the same therapies[33,34,35,36]. However, in 2021, the irreversible tyrosine kinase inhibitor, mobocertinib[37]was FDA approved for the treatment of NSCLC with EGFR exon 20 insertion mutations. Additionally, in 2022, the FDA granted breakthrough therapy designation to the irreversible EGFR inhibitors, CLN-081 (TPC-064)[38] and sunvozertinib[39], for locally advanced or metastatic non-small cell lung cancer harboring EGFR exon 20 insertion mutations. In lung cancer containing EGFR exon 19 or 21 activating mutations, treatment with TKIs is eventually associated with the emergence of drug resistance[40]. The primary resistance mutation that emerges following treatment with first-generation TKI is T790M, accounting for 50-60% of resistant cases[19]. Third generation TKIs were developed to maintain sensitivity in the presence of T790M. Osimertinib[41] (2015) is an irreversible inhibitor indicated for metastatic EGFR T790M positive lung cancer and for the first-line treatment of metastatic NSCLC containing EGFR exon 19 deletions or exon 21 L858R mutations. Like first-generation TKIs, treatment with osimertinib is associated with acquired resistance. In this case, resistance is associated with the C797S mutation and occurs in 22-44% of cases[40]. The T790M and C797S mutations may be each selected following sequential treatment with a first-generation TKI followed by a third-generation TKI or vice versa[42]. T790M and C797S can occur in either cis or trans allelic orientation[42]. If C797S is observed following progression after treatment with a third-generation TKI in the first-line setting, sensitivity may be retained to first-generation TKIs[42]. If C797S co-occurs in trans with T790M following sequential treatment with first- and third-generation TKIs, patients may exhibit sensitivity to combination first- and third-generation

Biomarker Descriptions (continued)

TKIs, but resistance to third-generation TKIs alone[42,43]. However, C797S occurring in cis conformation with T790M, confers resistance to first- and third-generation TKIs[42]. Fourth-generation TKIs are in development to overcome acquired C797S and T790M resistance mutations after osimertinib treatment. EGFR targeting antibodies including cetuximab (2004), panitumumab (2006), and necitumumab (2016) are under investigation in combination with EGFR-targeting TKIs for efficacy against EGFR mutations. The bispecific antibody, amivantamab[44], targeting EGFR and MET was approved (2021) for NSCLC tumors harboring EGFR exon 20 insertion mutations. CPO301[45] received a fast track designation (2023) from the FDA for EGFR mutations in patients with metastatic NSCLC who are relapsed/refractory or ineligible for EGFR targeting therapy such as 3rd-generation EGFR inhibitors including osimertinib. The Oncoprex immunogene therapy quaratusugene ozeplasmid[46] in combination with osimertinib received a fast track designation from the FDA (2020) for NSCLC tumors harboring EGFR mutations that progressed on osimertinib alone. BDTX-189[47] was granted a fast track designation (2020) for the treatment of solid tumors harboring an EGFR exon 20 insertion mutation.

PIK3CA (phosphatidylinositol-4,5-bisphosphate 3-kinase catalytic subunit alpha)

Background: The PIK3CA gene encodes the phosphatidylinositol-4,5-bisphosphate 3-kinase catalytic subunit alpha of the class I phosphatidylinositol 3-kinase (PI3K) enzyme[48]. PI3K is a heterodimer that contains a p85 regulatory subunit, which couples one of four p110 catalytic subunits to activated tyrosine protein kinases[49,50]. The p110 catalytic subunits include p110α, β, δ, γ and are encoded by genes PIK3CA, PIK3CB, PIK3CD, and PIK3CG, respectively[49]. PI3K catalyzes the conversion of phosphatidylinositol (4,5)-bisphosphate (PI(4,5)P2) into phosphatidylinositol (3,4,5)-trisphosphate (PI(3,4,5)P3) while the phosphatase and tensin homolog (PTEN) catalyzes the reverse reaction[51,52]. The reversible phosphorylation of inositol lipids regulates diverse aspects of cell growth and metabolism[51,52,53,54]. Recurrent somatic alterations in PIK3CA are frequent in cancer and result in the activation of PI3K/AKT/MTOR pathway, which can influence several hallmarks of cancer including cell proliferation, apoptosis, cancer cell metabolism and invasion, and genetic instability[55,56,57].

Alterations and prevalence: Recurrent somatic activating mutations in PIK3CA are common in diverse cancers and are observed in 20-30% of breast, cervical, and uterine cancers and 10-20% of bladder, gastric, head and neck, and colorectal cancers[10,11]. Activating mutations in PIK3CA commonly occur in exons 10 and 21 (previously referred to as exons 9 and 20 due to exon 1 being untranslated)[58,59]. These mutations typically cluster in the exon 10 helical (codons E542/E545) and exon 21 kinase (codon H1047) domains, each having distinct mechanisms of activation[60,61,62]. PIK3CA resides in the 3q26 cytoband, a region frequently amplified (10-30%) in diverse cancers including squamous carcinomas of the lung, cervix, head and neck, and esophagus, and in serous ovarian and uterine cancers[10,11].

Potential relevance: The PI3K inhibitor, alpelisib[63], is FDA approved (2019) in combination with fulvestrant for the treatment of patients with PIK3CA-mutated, hormone receptor (HR)-positive, human epidermal growth factor receptor 2 (HER2)-negative, advanced or metastatic breast cancer[64]. Additionally, a phase Ib study of alpelisib with letrozole in patients with metastatic estrogen receptor (ER)-positive breast cancer, the clinical benefit rate, defined as lack of disease progression ≥ 6 months, was 44% (7/16) in PIK3CA-mutated tumors and 20% (2/20) in PIK3CA wild-type tumors[65]. Specifically, exon 20 H1047R mutations were associated with more durable clinical responses in comparison to exon 9 E545K mutations[65]. However, alpelisib did not improve response when administered with letrozole in patients with ER+ early breast cancer with PIK3CA mutations[66]. Case studies with MTOR inhibitors sirolimus and temsirolimus report isolated cases of clinical response in PIK3CA mutated refractory cancers[67,68].

Relevant Therapy Summary

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types ✖ No evidence

EGFR exon 19 deletion

Relevant Therapy	FDA	NCCN	EMA	ESMO	Clinical Trials*
osimertinib	●	●	●	●	● (III)
erlotinib + ramucirumab	●	●	✖	●	✖
bevacizumab + erlotinib	✖	●	●	●	✖
bevacizumab (Allergan) + erlotinib	✖	✖	●	✖	✖
bevacizumab (Celltrion) + erlotinib	✖	✖	●	✖	✖

* Most advanced phase (IV, III, II/III, II, I/II, I) is shown and multiple clinical trials may be available.

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Relevant Therapy Summary (continued)

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types ✖ No evidence

EGFR exon 19 deletion (continued)

Relevant Therapy	FDA	NCCN	EMA	ESMO	Clinical Trials*
bevacizumab (Mabxience) + erlotinib	✖	✖	●	✖	✖
bevacizumab (Pfizer) + erlotinib	✖	✖	●	✖	✖
bevacizumab (Samsung Bioepis) + erlotinib	✖	✖	●	✖	✖
bevacizumab (Stada) + erlotinib	✖	✖	●	✖	✖
atezolizumab + bevacizumab + carboplatin + paclitaxel	✖	✖	✖	●	✖
gefitinib + carboplatin + pemetrexed	✖	✖	✖	●	✖
amivantamab, lazertinib, chemotherapy	✖	✖	✖	✖	● (III)
osimertinib, chemotherapy	✖	✖	✖	✖	● (III)
patritumab deruxtecan	✖	✖	✖	✖	● (III)
savolitinib, osimertinib	✖	✖	✖	✖	● (III)
osimertinib, savolitinib	✖	✖	✖	✖	● (II)
BLU-945, osimertinib	✖	✖	✖	✖	● (I/II)
chemotherapy	✖	✖	✖	✖	● (I/II)
sunvozertinib	✖	✖	✖	✖	● (I/II)
ABBV 400	✖	✖	✖	✖	● (I)
BAY-2927088	✖	✖	✖	✖	● (I)
TNO-155, nazartinib	✖	✖	✖	✖	● (I)

EGFR p.(T790M) c.2369C>T

Relevant Therapy	FDA	NCCN	EMA	ESMO	Clinical Trials*
osimertinib	●	●	●	●	✖
atezolizumab + bevacizumab + carboplatin + paclitaxel	✖	✖	✖	●	✖
osimertinib, chemotherapy	✖	✖	✖	✖	● (III)
savolitinib, osimertinib	✖	✖	✖	✖	● (III)
chemotherapy	✖	✖	✖	✖	● (II/III)
BLU-945, osimertinib	✖	✖	✖	✖	● (I/II)
sunvozertinib	✖	✖	✖	✖	● (I/II)
ABBV 400	✖	✖	✖	✖	● (I)

* Most advanced phase (IV, III, II/III, II, I/II, I) is shown and multiple clinical trials may be available.

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Relevant Therapy Summary (continued)

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types ✖ No evidence

PIK3CA p.(H1047R) c.3140A>G

Relevant Therapy	FDA	NCCN	EMA	ESMO	Clinical Trials*
alpelisib + fulvestrant	○	○	○	○	✖
inavolisib	✖	✖	✖	✖	● (II)

EGFR p.(C797S) c.2389T>A

Relevant Therapy	FDA	NCCN	EMA	ESMO	Clinical Trials*
chemotherapy	✖	✖	✖	✖	● (II/III)
BLU-945, osimertinib	✖	✖	✖	✖	● (I/II)
sunvozertinib	✖	✖	✖	✖	● (I/II)
ABBV 400	✖	✖	✖	✖	● (I)

* Most advanced phase (IV, III, II/III, II, I/II, I) is shown and multiple clinical trials may be available.

Relevant Therapy Details

Current FDA Information

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types

FDA information is current as of 2023-08-16. For the most up-to-date information, search www.fda.gov.

EGFR exon 19 deletion

● erlotinib + ramucirumab

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2022-03-22 **Variant class:** EGFR exon 19 deletion

Indications and usage:

CYRAMZA® is a human vascular endothelial growth factor receptor 2 (VEGFR2) antagonist indicated:

- as a single agent or in combination with paclitaxel, for treatment of advanced or metastatic gastric or gastro-esophageal junction adenocarcinoma with disease progression on or after prior fluoropyrimidine- or platinum-containing chemotherapy.
- in combination with erlotinib, for first-line treatment of metastatic non-small cell lung cancer with epidermal growth factor receptor (EGFR) exon 19 deletions or exon 21 (L858R) mutations.
- in combination with docetaxel, for treatment of metastatic non-small cell lung cancer with disease progression on or after platinum-based chemotherapy. Patients with EGFR or ALK genomic tumor aberrations should have disease progression on FDA-approved therapy for these aberrations prior to receiving CYRAMZA®.
- in combination with FOLFIRI, for the treatment of metastatic colorectal cancer with disease progression on or after prior therapy with bevacizumab, oxaliplatin, and a fluoropyrimidine.
- as a single agent, for the treatment of hepatocellular carcinoma in patients who have an alpha fetoprotein of ≥400 ng/mL and have been treated with sorafenib.

Reference:

https://www.accessdata.fda.gov/drugsatfda_docs/label/2022/125477s042lbl.pdf

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EGFR exon 19 deletion (continued)

osimertinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-06-21 **Variant class:** EGFR exon 19 deletion

Indications and usage:

TAGRISSO® is a kinase inhibitor indicated for:

- as adjuvant therapy after tumor resection in adult patients with non-small cell lung cancer (NSCLC) whose tumors have epidermal growth factor receptor (EGFR) exon 19 deletions or exon 21 L858R mutations, as detected by an FDA-approved test
- the first-line treatment of adult patients with metastatic NSCLC whose tumors have EGFR exon 19 deletions or exon 21 L858R mutations, as detected by an FDA-approved test.
- the treatment of adult patients with metastatic EGFR T790M mutation-positive NSCLC, as detected by an FDA-approved test, whose disease has progressed on or after EGFR TKI therapy.

Reference:

https://www.accessdata.fda.gov/drugsatfda_docs/label/2023/208065Orig1s028lbl.pdf

EGFR p.(T790M) c.2369C>T

osimertinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-06-21 **Variant class:** EGFR T790M mutation

Indications and usage:

TAGRISSO® is a kinase inhibitor indicated for:

- as adjuvant therapy after tumor resection in adult patients with non-small cell lung cancer (NSCLC) whose tumors have epidermal growth factor receptor (EGFR) exon 19 deletions or exon 21 L858R mutations, as detected by an FDA-approved test
- the first-line treatment of adult patients with metastatic NSCLC whose tumors have EGFR exon 19 deletions or exon 21 L858R mutations, as detected by an FDA-approved test.
- the treatment of adult patients with metastatic EGFR T790M mutation-positive NSCLC, as detected by an FDA-approved test, whose disease has progressed on or after EGFR TKI therapy.

Reference:

https://www.accessdata.fda.gov/drugsatfda_docs/label/2023/208065Orig1s028lbl.pdf

PIK3CA p.(H1047R) c.3140A>G

alpelisib + fulvestrant

Cancer type: Breast Cancer **Label as of:** 2022-11-22 **Variant class:** PIK3CA H1047R mutation

Other criteria: ERBB2 negative, Hormone receptor positive

Indications and usage:
PIQRAY® is a kinase inhibitor indicated in combination with fulvestrant for the treatment of postmenopausal women, and men, with hormone receptor (HR)- positive, human epidermal growth factor receptor 2 (HER2)-negative, PIK3CA-mutated, advanced or metastatic breast cancer as detected by an FDA-approved test following progression on or after an endocrine-based regimen.

Reference:

https://www.accessdata.fda.gov/drugsatfda_docs/label/2022/212526s007lbl.pdf

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Current NCCN Information

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types

NCCN information is current as of 2023-08-01. For the most up-to-date information, search www.nccn.org.
For NCCN International Adaptations & Translations, search www.nccn.org/global/what-we-do/international-adaptations.

Some variant specific evidence in this report may be associated with a broader set of alterations from the NCCN Guidelines. Specific variants listed in this report were sourced from approved therapies or scientific literature. These therapeutic options are appropriate for certain population segments with cancer. Refer to the NCCN Guidelines® for full recommendation.

EGFR exon 19 deletion + EGFR p.(T790M) c.2369C>T

● osimertinib

Cancer type: Non-Small Cell Lung Cancer **Variant classes:** EGFR T790M mutation & EGFR exon 19 deletion

NCCN Recommendation category: 1

Population segment (Line of therapy):
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic, Progression, Symptomatic, Asymptomatic (Subsequent therapy)

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2023]

EGFR exon 19 deletion

● osimertinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR exon 19 deletion

NCCN Recommendation category: 1

Population segment (Line of therapy):
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic, Biomarker discovered prior to first line therapy (First-line therapy); Preferred intervention

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2023]

● bevacizumab + erlotinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR exon 19 deletion

NCCN Recommendation category: 2A

Population segment (Line of therapy):
- Non-squamous Cell; Advanced, Metastatic, Biomarker discovered prior to first line therapy (First-line therapy); Other recommended intervention
- Non-squamous Cell; Advanced, Metastatic, Biomarker discovered during first line therapy (First-line therapy)

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2023]

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

EGFR exon 19 deletion (continued)

erlotinib + ramucirumab

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR exon 19 deletion

NCCN Recommendation category: 2A

Population segment (Line of therapy):
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic, Biomarker discovered prior to first line therapy (First-line therapy); Other recommended intervention
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic, Biomarker discovered during first line therapy (First-line therapy)

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2023]

osimertinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR exon 19 deletion

NCCN Recommendation category: 2A

Population segment (Line of therapy):
- Brain Metastases, Leptomeningeal Metastases (Line of therapy not specified); Preferred intervention

Reference: NCCN Guidelines® - NCCN-Central Nervous System Cancers [Version 1.2023]

osimertinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR exon 19 deletion

NCCN Recommendation category: 2A

Population segment (Line of therapy):
- Stage IB , Stage IIA, Stage IIB, Stage IIIA, Stage IIIB; Resected (Adjuvant therapy)
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic, Biomarker discovered during first line therapy (First-line therapy); Preferred intervention
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic, Progression, Symptomatic, Asymptomatic (Subsequent therapy)
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Leptomeningeal Metastases, Progression (Subsequent therapy); Consider

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2023]

EGFR p.(T790M) c.2369C>T

osimertinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

NCCN Recommendation category: 1

Population segment (Line of therapy):
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic, Progression, Asymptomatic, Symptomatic (Subsequent therapy)

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2023]

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

EGFR p.(T790M) c.2369C>T (continued)

osimertinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

NCCN Recommendation category: 2A

Population segment (Line of therapy):
- Brain Metastases, Leptomeningeal Metastases (Line of therapy not specified); Preferred intervention

Reference: NCCN Guidelines® - NCCN-Central Nervous System Cancers [Version 1.2023]

PIK3CA p.(H1047R) c.3140A>G

alpelisib + fulvestrant

Cancer type: Breast Cancer **Variant class:** PIK3CA activating mutation

Other criteria: ERBB2 negative, Hormone receptor positive

NCCN Recommendation category: 1

Population segment (Line of therapy):
- Stage IV; Recurrent, Unresectable, Local, Regional, Invasive (Second-line therapy, Subsequent therapy); Preferred intervention

Reference: NCCN Guidelines® - NCCN-Breast Cancer [Version 4.2023]

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

Current EMA Information

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types

EMA information is current as of 2023-08-16. For the most up-to-date information, search www.ema.europa.eu/ema.

EGFR exon 19 deletion

● bevacizumab (Allergan) + erlotinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-01-05 **Variant class:** EGFR exon 19 deletion

Reference:
https://www.ema.europa.eu/en/documents/product-information/mvasi-epar-product-information_en.pdf

● bevacizumab (Celltrion) + erlotinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-05-10 **Variant class:** EGFR exon 19 deletion

Reference:
https://www.ema.europa.eu/en/documents/product-information/vegzelma-epar-product-information_en.pdf

● bevacizumab (Mabxience) + erlotinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-07-26 **Variant class:** EGFR exon 19 deletion

Reference:
https://www.ema.europa.eu/en/documents/product-information/alymsys-epar-product-information_en.pdf

● bevacizumab (Pfizer) + erlotinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-01-05 **Variant class:** EGFR exon 19 deletion

Reference:
https://www.ema.europa.eu/en/documents/product-information/zirabev-epar-product-information_en.pdf

● bevacizumab (Samsung Bioepis) + erlotinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-04-11 **Variant class:** EGFR exon 19 deletion

Reference:
https://www.ema.europa.eu/en/documents/product-information/aybintio-epar-product-information_en.pdf

● bevacizumab (Samsung Bioepis) + erlotinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-04-11 **Variant class:** EGFR exon 19 deletion

Reference:
https://www.ema.europa.eu/en/documents/product-information/onbevzi-epar-product-information_en.pdf

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

EGFR exon 19 deletion (continued)

bevacizumab (Stada) + erlotinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-07-14 **Variant class:** EGFR exon 19 deletion

Reference:
https://www.ema.europa.eu/en/documents/product-information/oyavas-epar-product-information_en.pdf

bevacizumab + erlotinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2022-12-15 **Variant class:** EGFR exon 19 deletion

Reference:
https://www.ema.europa.eu/en/documents/product-information/abevmy-epar-product-information_en.pdf

bevacizumab + erlotinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-03-17 **Variant class:** EGFR exon 19 deletion

Reference:
https://www.ema.europa.eu/en/documents/product-information/avastin-epar-product-information_en.pdf

osimertinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-08-11 **Variant class:** EGFR exon 19 deletion

Reference:
https://www.ema.europa.eu/en/documents/product-information/tagrisso-epar-product-information_en.pdf

EGFR p.(T790M) c.2369C>T

osimertinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-08-11 **Variant class:** EGFR T790M mutation

Reference:
https://www.ema.europa.eu/en/documents/product-information/tagrisso-epar-product-information_en.pdf

PIK3CA p.(H1047R) c.3140A>G

alpelisib + fulvestrant

Cancer type: Breast Cancer **Label as of:** 2023-02-20 **Variant class:** PIK3CA H1047R mutation

Other criteria: ERBB2 negative, Hormone receptor positive

Reference:
https://www.ema.europa.eu/en/documents/product-information/piqray-epar-product-information_en.pdf

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

Current ESMO Information

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types

ESMO information is current as of 2023-08-01. For the most up-to-date information, search www.esmo.org.

EGFR exon 19 deletion

● osimertinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR exon 19 deletion

ESMO Level of Evidence/Grade of Recommendation: I / A

Population segment (Line of therapy):
- Stage IB , Stage IIA, Stage IIB, Stage IIIA; Resected (Adjuvant therapy); ESMO-MCBS v1.1 score: A

Reference: ESMO Clinical Practice Guidelines - ESMO-Early-Stage and Locally Advanced (non-metastatic) Non-Small-Cell Lung Cancer [Ann Oncol (2017) 28 (suppl 4): iv1–iv21. (eUpdate: 01 September 2021, 04 May 2020)]

● osimertinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR exon 19 deletion

ESMO Level of Evidence/Grade of Recommendation: I / A

Population segment (Line of therapy):
- Stage IV; Advanced, Metastatic (First-line therapy); ESMO-MCBS v1.1 score: 4

Reference: ESMO Clinical Practice Guidelines - ESMO-Oncogene-addicted Metastatic Non-Small-Cell Lung Cancer [Annals of Oncology (2023), doi: https://doi.org/10.1016/j.annonc.2022.12.009 (Published)]

● bevacizumab + erlotinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR exon 19 deletion

ESMO Level of Evidence/Grade of Recommendation: I / B

Population segment (Line of therapy):
- Stage IV; Advanced, Metastatic (First-line therapy); ESMO-MCBS v1.1 score: 2

Reference: ESMO Clinical Practice Guidelines - ESMO-Oncogene-addicted Metastatic Non-Small-Cell Lung Cancer [Annals of Oncology (2023), doi: https://doi.org/10.1016/j.annonc.2022.12.009 (Published)]

● erlotinib + ramucirumab

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR exon 19 deletion

ESMO Level of Evidence/Grade of Recommendation: I / B

Population segment (Line of therapy):
- Stage IV; Advanced, Metastatic (First-line therapy); ESMO-MCBS v1.1 score: 3

Reference: ESMO Clinical Practice Guidelines - ESMO-Oncogene-addicted Metastatic Non-Small-Cell Lung Cancer [Annals of Oncology (2023), doi: https://doi.org/10.1016/j.annonc.2022.12.009 (Published)]

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

EGFR exon 19 deletion (continued)

gefitinib + carboplatin + pemetrexed

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR exon 19 deletion

ESMO Level of Evidence/Grade of Recommendation: I / B

Population segment (Line of therapy):
- Stage IV; Advanced, Metastatic (First-line therapy)

Reference: ESMO Clinical Practice Guidelines - ESMO-Oncogene-addicted Metastatic Non-Small-Cell Lung Cancer [Annals of Oncology (2023), doi: https://doi.org/10.1016/j.annonc.2022.12.009 (Published)]

atezolizumab + bevacizumab + carboplatin + paclitaxel

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR exon 19 deletion

ESMO Level of Evidence/Grade of Recommendation: III / B

Population segment (Line of therapy):
- Stage IV; Advanced, Metastatic, Progression (Subsequent therapy); ESMO-MCBS v1.1 score: 3

Reference: ESMO Clinical Practice Guidelines - ESMO-Oncogene-addicted Metastatic Non-Small-Cell Lung Cancer [Annals of Oncology (2023), doi: https://doi.org/10.1016/j.annonc.2022.12.009 (Published)]

EGFR p.(T790M) c.2369C>T

osimertinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

ESMO Level of Evidence/Grade of Recommendation: I / A

Population segment (Line of therapy):
- Stage IV; Advanced, Metastatic (Second-line therapy); ESMO-MCBS v1.1 score: 4

Reference: ESMO Clinical Practice Guidelines - ESMO-Oncogene-addicted Metastatic Non-Small-Cell Lung Cancer [Annals of Oncology (2023), doi: https://doi.org/10.1016/j.annonc.2022.12.009 (Published)]

atezolizumab + bevacizumab + carboplatin + paclitaxel

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

ESMO Level of Evidence/Grade of Recommendation: III / B

Population segment (Line of therapy):
- Stage IV; Progression, Advanced, Metastatic (Subsequent therapy); ESMO-MCBS v1.1 score: 3

Reference: ESMO Clinical Practice Guidelines - ESMO-Oncogene-addicted Metastatic Non-Small-Cell Lung Cancer [Annals of Oncology (2023), doi: https://doi.org/10.1016/j.annonc.2022.12.009 (Published)]

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

PIK3CA p.(H1047R) c.3140A>G

alpelisib + fulvestrant

Cancer type: Breast Cancer **Variant class:** PIK3CA exon 20 mutation

Other criteria: ERBB2 negative, ER positive

ESMO Level of Evidence/Grade of Recommendation: I / B

Population segment (Line of therapy):
- Luminal A; Advanced, Metastatic (Second-line therapy); ESMO-MCBS v1.1 score: 2

Reference: ESMO Clinical Practice Guidelines - ESMO-Metastatic Breast Cancer [Ann Oncol (2021) VOLUME 32, ISSUE 12, P1475-1495, DECEMBER 01, 2021; DOI:https://doi.org/10.1016/j.annonc.2021.09.019]

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

Clinical Trials in Taiwan region:

Clinical Trials Summary

EGFR p.(T790M) c.2369C>T + EGFR p.(C797S) c.2389T>A

NCT ID	Title	Phase
NCT04862780	A Phase I/II Study Targeting Acquired Resistance Mechanisms in Patients With EGFR Mutant Non-Small Cell Lung Cancer.	I/II

EGFR exon 19 deletion + EGFR p.(T790M) c.2369C>T

NCT ID	Title	Phase
NCT04351555	A Phase III, Randomised, Controlled, Multi-center, 3-Arm Study of Neoadjuvant Osimertinib as Monotherapy or in Combination With Chemotherapy Versus Standard of Care Chemotherapy Alone for the Treatment of Patients With Epidermal Growth Factor Receptor Mutation Positive, Resectable Non-small Cell Lung Cancer	III

EGFR exon 19 deletion

NCT ID	Title	Phase
NCT04988295	A Phase III, Open-Label, Randomized Study of Amivantamab and Lazertinib in Combination With Platinum-Based Chemotherapy Compared With Platinum-Based Chemotherapy in Patients With EGFR-Mutated Locally Advanced or Metastatic Non-Small Cell Lung Cancer After Osimertinib Failure	III
NCT05120349	A Phase III, Double-blind, Randomised, Placebo-Controlled, International Study to Assess the Efficacy and Safety of Adjuvant Osimertinib Versus Placebo in Participants With EGFR Mutation-positive Stage IA2-IA3 Non-small Cell Lung Cancer, Following Complete Tumour Resection	III
NCT05338970	HERTHENA–Lung02: Phase III, Randomized, Open-label Study of Patritumab Deruxtecan Versus Platinum-Based Chemotherapy in Metastatic or Locally Advanced Non-Small Cell Lung Cancer (NSCLC) With Epidermal Growth Factor Receptor (EGFRm) Mutation After Failure treatment with epidermal growth factor (EGFR) tyrosine kinase inhibitors (TKIs)	III
NCT05261399	A Phase III, Randomised, Open-Label Study of Savolitinib in Combination With Osimertinib Versus Platinum-Based Doublet Chemotherapy in Participants With EGFR Mutated, MET-Overexpressed and/or Amplified, Locally Advanced or Metastatic Non-Small Cell Lung Cancer Who Have Progressed on Treatment With Osimertinib (SAFFRON).	III
NCT03778229	A Phase II Study Assessing the Efficacy of Osimertinib in Combination With Savolitinib in Patients With EGFRm+ and MET+, Locally Advanced or Metastatic Non Small Cell Lung Cancer Who Have Progressed Following Treatment With Osimertinib.	II
NCT04862780	A Phase I/II Study Targeting Acquired Resistance Mechanisms in Patients With EGFR Mutant Non-Small Cell Lung Cancer.	I/II
NCT05099172	An Open Label, First-in-human Study of BAY 2927088 in Participants With Advanced Non-small Cell Lung Cancer (NSCLC) Harboring an EGFR and/or HER2 Mutation	I
NCT03114319	An Open-label, Multi-center, Phase I, Dose Finding Study of Oral TNO155 in Adult Patients With Advanced Solid Tumors.	I
NCT05241873	Phase I/II Study of BLU-451 in Advanced Cancers With EGFR Exon 20 Insertion Mutations	I/II
NCT03974022	A Phase I/II, Open-Label, Multicenter Study to Assess the Safety, Tolerability, Pharmacokinetics and Anti-tumor Efficacy of DZD9008 in Patients With Advanced Non-Small Cell Lung Cancer (NSCLC) with EGFR or HER2 Mutation	I/II
NCT05029882	A Phase I First in Human Study Evaluating Safety, Pharmacokinetics and Efficacy of ABBV-400 in Adult Subjects With Advanced Solid Tumors	I

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

Clinical Trials Summary (continued)

EGFR p.(T790M) c.2369C>T

NCT ID	Title	Phase
NCT05261399	A Phase III, Randomised, Open-Label Study of Savolitinib in Combination With Osimertinib Versus Platinum-Based Doublet Chemotherapy in Participants With EGFR Mutated, MET-Overexpressed and/or Amplified, Locally Advanced or Metastatic Non-Small Cell Lung Cancer Who Have Progressed on Treatment With Osimertinib (SAFFRON).	III
NCT03178552	A Phase II/III Multicenter Study Evaluating the Efficacy and Safety of Multiple Targeted Therapies as Treatments for Patients With Advanced or Metastatic Non-Small Cell Lung Cancer (NSCLC) Harboring Actionable Somatic Mutations Detected in Blood (B-FAST: Blood-First Assay Screening Trial)	II/III
NCT05241873	Phase I/II Study of BLU-451 in Advanced Cancers With EGFR Exon 20 Insertion Mutations	I/II
NCT03974022	A Phase I/II, Open-Label, Multicenter Study to Assess the Safety, Tolerability, Pharmacokinetics and Anti-tumor Efficacy of DZD9008 in Patients With Advanced Non-Small Cell Lung Cancer (NSCLC) with EGFR or HER2 Mutation	I/II
NCT05029882	A Phase I First in Human Study Evaluating Safety, Pharmacokinetics and Efficacy of ABBV-400 in Adult Subjects With Advanced Solid Tumors	I

PIK3CA p.(H1047R) c.3140A>G

NCT ID	Title	Phase
NCT04589845	Tumor-Agnostic Precision Immunooncology and Somatic Targeting Rational for You (TAPISTRY) Phase II Platform Trial	II

EGFR p.(C797S) c.2389T>A

NCT ID	Title	Phase
NCT03178552	A Phase II/III Multicenter Study Evaluating the Efficacy and Safety of Multiple Targeted Therapies as Treatments for Patients With Advanced or Metastatic Non-Small Cell Lung Cancer (NSCLC) Harboring Actionable Somatic Mutations Detected in Blood (B-FAST: Blood-First Assay Screening Trial)	II/III
NCT05241873	Phase I/II Study of BLU-451 in Advanced Cancers With EGFR Exon 20 Insertion Mutations	I/II
NCT03974022	A Phase I/II, Open-Label, Multicenter Study to Assess the Safety, Tolerability, Pharmacokinetics and Anti-tumor Efficacy of DZD9008 in Patients With Advanced Non-Small Cell Lung Cancer (NSCLC) with EGFR or HER2 Mutation	I/II
NCT05029882	A Phase I First in Human Study Evaluating Safety, Pharmacokinetics and Efficacy of ABBV-400 in Adult Subjects With Advanced Solid Tumors	I

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

Alerts Informed By Public Data Sources

Current FDA Information

Contraindicated | Not recommended | Resistance | Breakthrough | Fast Track

FDA information is current as of 2023-08-16. For the most up-to-date information, search www.fda.gov.

EGFR exon 19 deletion

patritumab deruxtecan

Cancer type: Non-Small Cell Lung Cancer

Variant class: EGFRi sensitizing mutation or EGFR exon 19 deletion

Supporting Statement:
The FDA has granted Breakthrough Therapy Designation to a potential first-in-class HER3 directed antibody-drug conjugate, patritumab deruxtecan, for metastatic or locally advanced, EGFR-mutant non-small cell lung cancer.

Reference:

https://www.cancernetwork.com/view/fda-grants-breakthrough-therapy-status-to-patritumab-deruxtecan-for-egfr-metastatic-nsclc

CPO-301

Cancer type: Non-Small Cell Lung Cancer

Variant class: EGFR mutation

Supporting Statement:
The FDA has granted Fast Track Designation to a first-in-class antibody drug conjugate, CPO301, for EGFR mutations in patients with metastatic non-small cell lung cancer (NSCLC) who are relapsed/refractory to or ineligible for EGFR targeting therapy such as 3rd-generation EGFR inhibitors including Osimertinib.

Reference:

http://iis.aastocks.com/20230612/10770455-0.PDF

osimertinib + quaratusugene ozeplasmid

Cancer type: Non-Small Cell Lung Cancer

Variant class: EGFR mutation

Supporting Statement:
The FDA has granted Fast Track Designation to the immunogene therapy, quaratusugene ozeplasmid, in combination with EGFR inhibitor osimertinib for the treatment of EGFR mutations in non-small cell lung cancer (NSCLC) patients that progressed after treatment with osimertinib alone.

Reference:

https://www.genprex.com/news/genprex-receives-u-s-fda-fast-track-designation-for-gene-therapy-that-targets-lung-cancer/

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

EGFR p.(T790M) c.2369C>T

CPO-301

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

Supporting Statement:
The FDA has granted Fast Track Designation to a first-in-class antibody drug conjugate, CPO301, for EGFR mutations in patients with metastatic non-small cell lung cancer (NSCLC) who are relapsed/refractory to or ineligible for EGFR targeting therapy such as 3rd-generation EGFR inhibitors including Osimertinib.

Reference:

http://iis.aastocks.com/20230612/10770455-0.PDF

osimertinib + quaratusugene ozeplasmid

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

Supporting Statement:
The FDA has granted Fast Track Designation to the immunogene therapy, quaratusugene ozeplasmid, in combination with EGFR inhibitor osimertinib for the treatment of EGFR mutations in non-small cell lung cancer (NSCLC) patients that progressed after treatment with osimertinib alone.

Reference:

https://www.genprex.com/news/genprex-receives-u-s-fda-fast-track-designation-for-gene-therapy-that-targets-lung-cancer/

EGFR p.(C797S) c.2389T>A

CPO-301

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

Supporting Statement:
The FDA has granted Fast Track Designation to a first-in-class antibody drug conjugate, CPO301, for EGFR mutations in patients with metastatic non-small cell lung cancer (NSCLC) who are relapsed/refractory to or ineligible for EGFR targeting therapy such as 3rd-generation EGFR inhibitors including Osimertinib.

Reference:

http://iis.aastocks.com/20230612/10770455-0.PDF

osimertinib + quaratusugene ozeplasmid

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

Supporting Statement:
The FDA has granted Fast Track Designation to the immunogene therapy, quaratusugene ozeplasmid, in combination with EGFR inhibitor osimertinib for the treatment of EGFR mutations in non-small cell lung cancer (NSCLC) patients that progressed after treatment with osimertinib alone.

Reference:

https://www.genprex.com/news/genprex-receives-u-s-fda-fast-track-designation-for-gene-therapy-that-targets-lung-cancer/

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

Current NCCN Information

Contraindicated Not recommended Resistance Breakthrough Fast Track

NCCN information is current as of 2023-08-01. For the most up-to-date information, search www.nccn.org.
For NCCN International Adaptations & Translations, search www.nccn.org/global/what-we-do/international-adaptations.

Some variant specific evidence in this report may be associated with a broader set of alterations from the NCCN Guidelines. Specific variants listed in this report were sourced from approved therapies or scientific literature. These therapeutic options are appropriate for certain population segments with cancer. Refer to the NCCN Guidelines® for full recommendation.

EGFR p.(T790M) c.2369C>T

afatinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

Summary:
NCCN Guidelines® include the following supporting statement(s):
- "The most common known mechanism is the acquisition of T790M (which is a secondary mutation in EGFR), which renders the kinase resistant to erlotinib, gefitinib, dacomitinib, or afatinib."

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2023]

dacomitinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

Summary:
NCCN Guidelines® include the following supporting statement(s):
- "The most common known mechanism is the acquisition of T790M (which is a secondary mutation in EGFR), which renders the kinase resistant to erlotinib, gefitinib, dacomitinib, or afatinib."

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2023]

erlotinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

Summary:
NCCN Guidelines® include the following supporting statement(s):
- "The most common known mechanism is the acquisition of T790M (which is a secondary mutation in EGFR), which renders the kinase resistant to erlotinib, gefitinib, dacomitinib, or afatinib."

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2023]

gefitinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

Summary:
NCCN Guidelines® include the following supporting statement(s):
- "The most common known mechanism is the acquisition of T790M (which is a secondary mutation in EGFR), which renders the kinase resistant to erlotinib, gefitinib, dacomitinib, or afatinib."

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2023]

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

Current EMA Information

Contraindicated Not recommended Resistance Breakthrough Fast Track

EMA information is current as of 2023-08-16. For the most up-to-date information, search www.ema.europa.eu/ema.

EGFR p.(T790M) c.2369C>T

gefitinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-07-27 **Variant class:** EGFR T790M mutation

Reference:
https://www.ema.europa.eu/en/documents/product-information/gefitinib-mylan-epar-product-information_en.pdf

gefitinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2023-07-17 **Variant class:** EGFR T790M mutation

Reference:
https://www.ema.europa.eu/en/documents/product-information/iressa-epar-product-information_en.pdf

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.09(003).

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